

COMPREHENSIVE REVIEW

Mentalization-Based Therapy for Borderline Personality Disorder: State-Of-The-Science and Future Directions

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Received: 5 April 2025 | **Revised:** 25 May 2025 | **Accepted:** 27 June 2025

Keywords: borderline personality disorder | evidence | mentalization-based therapy | randomized controlled trials | review

ABSTRACT

Introduction: Mentalization-based therapy (MBT) is an evidence-based treatment for borderline personality disorder (BPD) and the second most studied psychotherapeutic intervention for this disorder. Despite its growing prominence, there is a lack of focused reviews summarizing the evidence base from randomized clinical trials (RCTs). This review aims to address this gap and highlight directions for future research.

Methods: This narrative review synthesizes findings from RCTs evaluating MBT for BPD. Building on the seven studies identified in the Cochrane review by Storebø et al. (2020), we conducted an updated search to incorporate RCTs published to 1st April 2025, expanding the evidence base to 14 trials. Eligible studies were assessed for effectiveness outcomes and methodological quality.

Results: While some trials report improvements in BPD symptoms and interpersonal functioning, significant limitations exist. Research has largely focused on symptom reduction, despite concerns about the validity of categorical PD diagnoses. The trials were predominantly conducted in European settings, involving primarily female participants, and did not sufficiently account for BPD severity, comorbidities, or developmental stages. All identified RCTs were at high risk of bias, with significant issues involving attention, attrition, and allegiance bias.

Conclusions: MBT shows promise, but methodological shortcomings and limited generalizability constrain its evidence base. Future research should prioritize rigorously designed trials with diverse samples, assess core personality dysfunction using dimensional models, and explore adaptations for adolescents, older adults, and comorbid mental disorders to improve and broaden the clinical utility of MBT for BPD.

1 | Introduction

Borderline personality disorder (BPD) is a severe mental disorder characterized by instability of affects, behaviours and interpersonal relationships (Leichsenring et al. 2024). BPD is furthermore associated with an increased risk of comorbidity, severe functional impairments, high suicidal risk, intensive use of treatment, poor treatment compliance and high societal costs (Hastrup et al. 2019; Leichsenring et al. 2024).

Several evidence-based treatments that target core features of BPD have been proposed, including mentalization-based treatment (MBT). MBT represents the second most studied approach for BPD, only surpassed by dialectical behaviour therapy (Storebø et al. 2020). Mentalization was initially proposed by Peter Fonagy (1989) and refers to the capacity to perceive and interpret oneself and others' behaviour in terms of underlying feelings, thoughts, needs, motives, desires and intentions (Bateman and Fonagy 2010). Mentalizing is a dynamic psychological

Summary

- Mentalization-based therapy (MBT) is a widely used treatment for borderline personality disorder (BPD), but evidence from randomized clinical trials (RCTs) remains mixed.
- Current MBT research primarily focuses on symptom reduction, despite concerns about the validity of categorical BPD diagnoses.
- The generalizability of MBT findings is limited, as most RCTs have been conducted in European settings with predominantly female participants.
- Methodological challenges, including high risk of bias and limited consideration of comorbidities and developmental stages, hinder the reliability of existing evidence.
- Future research should prioritize diverse samples, incorporate dimensional models of personality dysfunction, and explore MBT adaptations for adolescents, older adults, and individuals with comorbid mental disorders.

capacity that has evolved in human evolutionary history as a response to increasing social complexity. Rather than being a fixed trait, it involves state- and context-dependent processes and is shaped by interactions with others (Allen et al. 2008; Hill et al. 2023). Mentalization develops through early attachment relationships and is considered a core aspect of normative development (Fonagy and Target 1996). During early interactions, caregivers are expected to respond sensitively to the infant's expressive behaviours, adapting to its needs and desires. Through marked and contingent mirroring, caregivers signal their referential emotional displays, which help the child understand its own and others' subjective experiences, and support the child in internalizing emotional security, a sense of safety, trust, agency and empathy (Beebe and Lachmann 2013). Engagement in interactions with a mentalizing caregiver teaches the child that information from others is reliable, relevant, safe to trust, can be stored in memory and generalized to other contexts (Fonagy and Allison 2014). This fosters an openness to learning, conceptualized as epistemic trust, which facilitates the child's engagement in joint attention, 'we-mode', and meaning-making (Fonagy and Allison 2014; Fonagy and Target 1996; Sharp et al. 2020).

Inadequate development of mentalizing abilities is theorized to have significant negative consequences, potentially contributing to the emergence of various mental disorders. For example, mentalizing is considered a central factor in the aetiology of BPD, where childhood experiences with low parental mentalizing may influence the quality of future interpersonal relationships (Vogt and Norman 2019). BPD is associated with impaired mentalization, epistemic mistrust, difficulties in joint meaning-making and challenges in joint intentionality, particularly in emotionally intense situations within close relationships (Fonagy and Allison 2014). As a result, MBT has centred its approach on therapeutic interventions aimed at enhancing the patient's capacity to mentalize within the context

of attachment relationships, such as the therapist–patient dyad. Enhanced mentalizing is thought to improve emotion regulation, interpersonal functioning, social cognition and cognitive flexibility. The core of MBT interventions centres around a not-knowing stance, curiosity, openness and empathy. MBT targets mentalizing dysfunctions by utilizing interventions like emphatic validation, clarification, elaboration, confrontation, basic mentalization and relational mentalization (Bateman and Fonagy 2016).

Anthony Bateman and Peter Fonagy originally developed MBT as a structured treatment for BPD, combining individual therapy and group therapy within a day hospital setting, rooted in mentalization-based theory. The original program consisted of 18 months of therapy (Bateman and Fonagy 2016). Since then, MBT has been adapted and empirically tested in various settings, with modifications to its scope, duration and target population. For example, MBT has been tailored to address specific comorbidities, applied to adolescent populations and extended to caregivers. More recent iterations of MBT have incorporated social contextual factors, emphasizing the importance of not only working with individuals but also engaging with the broader social networks, including families, peers and professionals (Fuggle et al. 2024; Luyten et al. 2024). The growing interest in the mentalizing approach has positioned mentalization as a crucial human capacity, extending its relevance to the understanding of temporary or permanent impairments in mentalizing across various psychopathologies (Sharp and Bevington 2022; Luyten et al. 2020). Although MBT is rooted in psychodynamic theories, it is now widely regarded as an integrative approach compatible with and supplementing cognitive, systemic and psychoanalytic approaches (Sharp and Rossouw 2024). The concept of mentalization has been proposed as a common factor across multiple therapeutic methods and has been recognized as a core mechanism of change in treatment (Bateman et al. 2018; Nahum et al. 2019).

1.1 | The Evidence Based of MBT

The most recent Cochrane review on the effectiveness of psychotherapy treatment for BPD (Storebø et al. 2020) included seven randomized clinical trials (RCTs) evaluating MBT (Bateman and Fonagy 1999, 2009; Rossouw and Fonagy 2012; Jørgensen et al. 2013; Robinson et al. 2016; Laurensen et al. 2018; Philips et al. 2018). The review concluded that MBT was more effective than control interventions in reducing self-harm, suicidality and depression (Storebø et al. 2020). However, when considering all outcomes included in the meta-analyses, the results were less clear.

For *BPD severity*, six trials were included (Bateman and Fonagy 1999; Rossouw and Fonagy 2012; Jørgensen et al. 2013; Robinson et al. 2016; Laurensen et al. 2018; Philips et al. 2018), and while individuals in the MBT arms improved over time, MBT did not produce superior results compared to the control interventions at end of treatment (EOT) nor at follow-up periods 0–6 months after EOT (one trial) or above 12 months follow-up (two trials). Four trials reported data on *self-harm* (Bateman and Fonagy 1999, 2009; Philips et al. 2018; Rossouw and Fonagy 2012). While Philips et al. (2018) found

no superiority in reducing self-harm severity, the remaining trials, which reported dichotomous data, found MBT superior to control interventions at EOT. Only one trial provided follow-up data, which showed continued benefits. A similar pattern was observed for *suicide-related outcomes*: three trials (Bateman and Fonagy 1999, 2009; Philips et al. 2018) found MBT superior at EOT, but follow-up results were mixed—no superiority within 6 months, yet superiority in one trial beyond 12 months. Three trials (Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013) assessed *psychosocial functioning*, finding no superiority of MBT at EOT or follow-up. However, four trials (Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013; Philips et al. 2018) showed that MBT improved *interpersonal problems* at EOT, though this effect did not persist in follow-up (two trials). *Depression* was measured in five trials (Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013; Laurensen et al. 2018; Rossouw and Fonagy 2012), where MBT was non-superior to control interventions at EOT and with mixed findings in the follow-up period (non-superiority in the first 6 months, but not from 6 months to 1 year after EOT or above 1 year after EOT). *Attrition rates* were analysed in all seven trials, showing that MBT did not significantly reduce dropout rates compared to control interventions. *Serious adverse effects* were reported in only one trial (Robinson et al. 2016), while no trials examined several pre-specified Cochrane review outcomes, such as *anger*, *affective instability*, *chronic emptiness*, *impulsivity*, *abandonment fears*, *identity disturbance*, *dissociation* and *psychotic-like symptoms*. Thus, MBT's effectiveness in these areas remains unknown.

The Cochrane review used a random-effects statistical model, which is appropriate given the heterogeneity across studies but tends to yield more conservative estimates to mitigate random errors. Storebø et al. (2020) also applied a fixed-effects model, assuming greater homogeneity between studies. Under this model, MBT showed additional superiority over control interventions for BPD severity (beyond 12 months), psychosocial functioning (at EOT) and depression (at EOT and in follow-ups at 0–6 months and beyond 12 months).

In conclusion, Storebø et al.'s (2020) review provides a nuanced picture of MBT's effectiveness. While the random-effects model highlights some positive outcomes, fixed-effects analyses suggest additional benefits. However, the limitations of these models and the overall methodological constraints in existing RCTs underscore the need for further high-quality research.

1.2 | State-Of-The-Science: A Narrative Review

Previous reviews have examined the effectiveness of MBT for BPD (e.g., Volkert et al. 2019; Wolfs 2024), generally supporting its clinical utility. However, these reviews predate and/or do not include several recent RCTs. Given MBT's clinical and theoretical prominence, there is a need for an updated, focused review summarizing the current evidence base from RCTs. RCTs are considered the gold standard for evaluating the efficacy and effectiveness of psychological interventions. By minimizing biases through random allocation, rigorous control groups, and standardized outcome measures, RCTs provide the most reliable evidence for determining causality and assessing the impact

of therapeutic interventions (Bhude et al. 2018). While several non-randomized studies have highlighted the potential benefits of MBT, only RCTs can robustly address critical questions regarding its effectiveness compared to other treatments or standard care. Focusing on RCTs ensures high-quality evidence to evaluate MBT's effectiveness. This approach also helps to (1) *enhance clinical relevance*: By emphasizing controlled settings, RCTs provide insights into how MBT performs under standardized conditions, offering clear implications for clinical practice, (2) *enable comparisons across treatments*: RCTs often include direct comparisons between a given experimental intervention and other therapeutic modalities, facilitating an understanding of the relative or comparative benefits and (3) *identify limitations and gaps relevant for future research*: RCTs help pinpoint methodological challenges, guiding future research directions. By narrowing the scope to RCTs, this review aims to provide clinicians, researchers, and policymakers with a rigorous and focused evaluation of MBT's current evidence base and point to knowledge gaps relevant for future research.

To achieve this aim, a narrative state-of-the-science review design was utilized to synthesize current findings on the effectiveness of MBT, with a focus on evidence derived from RCTs. A state-of-the-science review is widely recognized for its ability to provide a solid state of current knowledge within a specific field or phenomenon (Sukhera 2022). This approach is particularly valuable as it allows for the exploration of focused research questions tailored to a specific audience, offering insights into the current status of a particular therapeutic approach, its development and future directions (Sukhera 2022). In this review, the state-of-the-science review design is employed to examine MBT as a treatment modality specifically for BPD, synthesizing findings from RCTs to assess its effectiveness and highlight areas for future development. In addition to assessing the overall effectiveness of MBT, we also aim to examine the outcomes used in the trials to measure effectiveness. By gaining an overview of the outcomes assessed across the trials, we aim to identify which variables of effectiveness have been explored and where there are gaps in our knowledge, where further research is needed to build a more comprehensive understanding of MBT's impact.

While narrative reviews do not provide definite answers, the design facilitates a nuanced discussion aimed at engaging practitioners and researchers. The goal is to generate insights not only into the current evidence base for MBT for BPD but also into how research methodologies can be refined to better evaluate its impact. By offering a detailed and interpretive synthesis of the evidence, the narrative review serves as a foundation for establishing new perspectives and guiding future research (Greenhalgh et al. 2018).

2 | Methods

2.1 | Search Strategy

The search strategy for this review was based on the comprehensive systematic review conducted by Storebø et al. (2020), which examined psychotherapeutic interventions for BPD, including MBT. To ensure the review includes the most up-to-date

evidence, we supplemented the studies identified by Storebø et al. (2020) with an updated search for RCTs published from January 2020 to 1 April 2025. The top-up search was conducted in the following electronic databases:

- PubMed/MEDLINE
- PsycINFO
- Embase

The search strategy utilized a combination of keywords and MeSH terms, including ‘mentalization-based therapy’, ‘borderline personality disorder’, ‘randomized controlled trial’, and related terms.

2.2 | Eligibility Criteria

Studies were eligible for inclusion if they met the following criteria:

1. The study was a randomized clinical trial.
2. The study evaluated the effectiveness of MBT in individuals diagnosed with BPD based on established diagnostic criteria (DSM or ICD).
3. The study reported quantitative outcomes.

Studies included in the Cochrane review were directly extracted from the original publication, while newly identified studies were screened by two reviewers for relevance and eligibility. Any discrepancies were resolved through discussion or consultation with a third reviewer. By leveraging the work of Storebø et al. (2020) and extending the search period, this review aims to provide a comprehensive synthesis of the most current evidence on the effectiveness of MBT for BPD.

3 | Results

We identified 14 RCTs investigating the effectiveness of MBT for BPD. In the following, we will provide an overview of these trials, which include RCTs focused on BPD, MBT for BPD with comorbidities, MBT for adolescents with BPD and MBT for family members and close ones of individuals with BPD. A summary of these studies is presented in Table 1, which provides an overview of the current evidence base, including study characteristics and key findings, organized in chronological order based on their publication date. We will further elaborate on the strengths and limitations of these studies and evaluate the quality of the evidence.

The first RCT of MBT was conducted by Bateman and Fonagy (1999). Since then, MBT has been tested in various settings and across different durations and levels of intensity. Even though BPD is commonly associated with various comorbid mental disorders, only three RCTs have specifically addressed this: one trial focused on MBT for BPD and comorbid eating disorders (Robinson et al. 2016), another investigated MBT for BPD with co-occurring substance use disorders (Philips et al. 2018) and a third tested MBT for BPD

and antisocial personality disorder (Mohajerin et al. 2024). Furthermore, only three RCTs to date have investigated the effectiveness of MBT for adolescents with BPD pathology compared to control treatments (Rossouw and Fonagy 2012; Beck et al. 2020; Mohajerin et al. 2025), but additional studies are currently underway (e.g., Remeus et al. 2024). Additionally, while substantial caregiver burden has been documented among relatives of both adults (Bailey and Grenyer 2013) and adolescents with BPD (Jørgensen, Storebø, Poulsen, and Simonsen 2021), only one trial has targeted family members of individuals with BPD (Bateman and Fonagy 2019).

To summarize, the results across the identified trials are somewhat mixed. While several studies indicate positive outcomes in MBT, the evidence base remains limited, particularly for MBT in the context of comorbidities, adolescents and family-focused interventions. The variability in trial designs, sample sizes and use of outcomes further complicates drawing definitive conclusions.

3.1 | Quality of the Evidence

The most recent Cochrane review summarizing the evidence for psychological therapies for BPD (Storebø et al. 2020) included seven trials of MBT. Below, we will outline the risk of bias associated with these trials based on the findings from the Cochrane review. Additionally, since the publication of the Cochrane review, seven additional trials have been conducted (Bateman and Fonagy 2019; Beck et al. 2020; Carlyle et al. 2020; Juul et al. 2023; Mohajerin et al. 2024; Mohajerin et al. 2025; Smits et al. 2020). The Beck et al. (2020) trial was subsequently carefully assessed in another systematic review related to the Cochrane review that focused on adolescents (Jørgensen, Storebø, Stoffers-Winterling, et al. 2021). To our knowledge, the remaining trials have not been assessed in a systematic review. Therefore, we will highlight the risk of bias we have identified for these trials.

Cochrane's tool for assessing risk of bias (Higgins et al. 2011) is used to categorize trials into three categories: low risk of bias, unclear risk of bias, or high risk of bias, according to guidelines provided in the Cochrane Handbook for Systematic Reviews of Interventions (Higgins et al. 2024). The tool assesses several types of bias, including selection bias, performance bias, detection bias, attrition bias, reporting bias and other biases (e.g., adherence bias, attention bias, and allegiance bias). If a trial has one or more domains with unclear or high risk of bias, then the trial is considered a high-risk-of-bias trial, and the quality of the evidence is downgraded because one cannot be certain that the effect estimates are true, and by downgrading the quality of the evidence, we ensure a more accurate assessment of the credibility of the evidence, highlighting the need for caution in interpreting the results (Higgins et al. 2024). Risk of bias assessments for the MBT trials are presented in Table 2.

All the trials of MBT for BPD are high-risk-of-bias trials, significantly impacting effect estimates and the overall quality of evidence. Notable biases include attention bias, attrition bias and allegiance bias. Future studies of MBT should conduct more rigorously designed trials with larger sample sizes. Addressing the current biases and limitations will be essential to provide more

TABLE 1 | Overview of trials on mentalization-based therapy for borderline personality disorder.

Trial	Country	Setting	n	Age	Gender	MBT content	Control	Duration	Key findings
Bateman and Fonagy 1999	UK	Day hospital	38 (19 in MBT)	31.8	57.9% female	– Weekly individual therapy (1×/week)—Group analytic therapy (3×/week, 1 h each)—Expressive therapy with psychodrama (1×/week, 1 h)—Weekly community meeting (1 h)—Monthly case administrator meeting (1 h)—Monthly psychiatrist consultation for medication review	Standard psychiatric care	18 months	– MBT led to superior outcomes, especially in the last 6 months, with maintained benefits at follow-up (Bateman and Fonagy 2001, 2008)—MBT was more cost-effective than TAU (Bateman and Fonagy 2003)
Bateman and Fonagy 2009	UK	Outpatient	134 (71 in MBT)	31.3	79.9% female	– Weekly combined individual and group psychotherapy—Both treatment arms: crisis contact/plans, pharmacotherapy, psychiatric review, and written treatment information	Structured clinical management	18 months	– Both treatments were effective, but MBT was superior to SCM on most outcomes at end of treatment and follow-up (Bateman et al. 2021)
Jørgensen et al. 2013	Denmark	Outpatient	111 (74 in MBT)	29.1	95–96% female	– Weekly individual therapy (45 min) and group therapy (1.5 h) over 18–20 months—Both treatment arms: Monthly psychoeducational program for 6 months	Supportive group treatment	18–20 months	– Both treatment groups showed sustained significant symptom changes after EOT, but MBT was not superior to SGT on any outcome except therapist-rated GAF (unblinded)—Treatment effects were sustained in follow-up, with no between-group differences (Jørgensen et al. 2014)
Laurensen et al. 2018	The Netherlands	Day hospital	95 (54 in MBT-DH)	34	79% female	Structured day hospital program (up to 18 months, 5 days/week, ~6 h/day)—Daily group psychotherapy—Weekly individual psychotherapy—Twice-weekly art therapy—Mentalizing cognitive group therapy—Writing therapy—Weekly social hour and community meeting—Psychiatric consultations available upon request	Specialist treatment as usual	6 months	– Both treatments showed significant improvements on all outcome variables with no between-group differences—Drop-out rates were significantly higher in S-TAU (34%) compared to MBT-DH (9%)

(Continues)

TABLE 1 | (Continued)

Trial	Country	Setting	n	Age	Gender	MBT content	Control	Duration	Key findings
Smits et al. 2020	The Netherlands	Outpatient and day hospital	114 (70 in MBT-DH)	29.9 in MBT-IOP, 31.4 in MBT-DH	80% in MBT-IOP, 84% in MBT-DH	– MBT-IOP: Two group therapy sessions per week—MBT-DH: Day hospital program (5 days/week)—5× weekly group psychotherapy—4× weekly art, writing, or mentalizing cognitive therapy—Weekly social hour and community meeting	Intensive outpatient MBT	18 months	– Significant improvements were observed for both treatments, with no between-group differences on most outcomes, except relational functioning where MBT-DH was superior—Treatment gains were maintained in the follow-up, with no significant between-group differences (Smits, Feenstra, et al. 2022)
Carlyle et al. 2020	New Zealand	Outpatient	72 (38 in MBT)	32.4 in MBT, 31.6 in SCM	100% female in MBT, 97.1% female in SCM	Weekly 1-h individual therapy and 1.5-h group therapy—First 12 weeks: Structured education on BPD and mentalization—Remaining 15 months: Modified group analytic approach—Case manager: Treatment/crisis plan and crisis intervention support	Structured clinical management	18 months	– No significant difference in self-harm rates between treatments, but differences emerged when subdividing into self-harm with serious suicidal intent (SA) and without (NSSH)—SA rates were higher in the MBT group, while NSSH rates were higher in the SCM group. Both groups showed a significant reduction (halved) in total rates of SA and NSSI
Juul et al. 2023	Denmark	Outpatient	166 (82 in long-term MBT, 84 in short-term MBT)	32.2 in long-term MBT, 31.7 in short-term MBT	92.7% female in long-term MBT, 89.1% female in short-term MBT	– Short-term MBT: 5 introductory sessions, 15 weekly group sessions, and biweekly individual therapy—Long-term MBT: 6 weeks of introduction, 12 months of weekly group therapy, and biweekly individual therapy	Long-term MBT was compared to short-term MBT	14 months versus 12 weeks	– MBT was equally effective on all outcomes when delivered with lower intensity, possibly expanding its accessibility to more individuals with BPD.

(Continues)

TABLE 1 | (Continued)

Trial	Country	Setting	<i>n</i>	Age	Gender	MBT content	Control	Duration	Key findings
MBT for BPD and specific comorbidities									
Robinson et al. 2016	UK	Outpatient	68 (34 in MBT-ED)	31.1	92.7% female	Weekly 50-min individual MBT-ED session—Weekly 90-min group MBT-ED with 1–2 therapists and 4–6 patients—Therapy duration: 12 months, with reassessment after 12 months and referral for further management if needed—Total: 44 individual and 44 group sessions	Specialist supportive clinical management for eating disorders	12 months	High dropout rates: 47% in MBT-ED and 37% in SSCM-ED, with only 15 participants (22%) completing the 18-month follow-up—Among completers, MBT-ED led to greater reduction in shape and weight concern compared to SSCM-ED—Both groups showed a decline in BPD symptoms over time—MBT-ED may improve body image psychopathology, but findings should be interpreted with caution due to high attrition rate
Philips et al. 2018	Sweden	Outpatient	46 (24 in MBT)	36.7	80.4% female	Combination of individual and group therapy. Mean of 63.3 MBT sessions (SD: 26.7, range 10–116).	Treatment as usual	15–18 months	No significant differences between groups on outcome variables—No suicide attempts in the MBT group, compared to four in TAU—Results are limited due to non-adherence to MBT principles by most therapists

(Continues)

TABLE 1 | (Continued)

Trial	Country	Setting	n	Age	Gender	MBT content	Control	Duration	Key findings
Mohajerin et al. 2024	Iran	Outpatient	108 (55 in MBT)	27.1	54.5% female in MBT, 56.6% female in UP	– Participants received 74–85 sessions (mean = 78.21, SD = 4.15)—60–67 sessions of group psychotherapy and 12–16 sessions of individual psychotherapy	Unified Protocol	17–20 months	– Both treatments led to short-term reductions in psychopathy traits, antisocial and BPD symptom severity, anger dysregulation, impulsivity, and self-harm—Symptoms relapsed almost completely in the follow-up, with UP showing more durable effects compared to MBT
MBT for adolescents									
Rossouw and Fonagy 2012	UK	Outpatient	80 (40 in MBT-A)	15.4 (MBT), 14.8 (TAU)	85% female	Weekly MBT—A individual sessions and monthly mentalization-based family therapy (MBT-F)	Treatment as usual	12 months	– 73% of the sample met diagnostic criteria for BPD despite it not being an inclusion criterion—MBT was more effective than TAU in reducing self-harm and depression, with improvements in mentalization, attachment avoidance and BPD symptoms and traits
Beck et al. 2020	Denmark	Outpatient	111 (55 in MBT-G)	15.8	99% female	Weekly MBT-G sessions for 1 year, five individual case formulation sessions, and six MBT-Parent sessions	Treatment as usual	12 months	– No differences between MBT-G and TAU on outcomes—High attrition rates in MBT (56%)—Lack of superiority of MBT-G persisted in the follow-up period (Jørgensen, Storebø, Bo, et al. 2021 ; Jørgensen et al. 2024)

(Continues)

TABLE 1 | (Continued)

Trial	Country	Setting	n	Age	Gender	MBT content	Control	Duration	Key findings
Mohajerin et al. 2025	Iran	Outpatient	91 (45 in MBT-A)	15.51 (MBT-A), 15.9 (UP-A)	68.9% female (MBT-A), 65.2% female (UP-A)	MBT-A weekly individual MBT-A sessions (50 min) plus a monthly session of family therapy for 1 year	Unified Protocol for Adolescents	12 months	– Both interventions significantly reduced BPD symptom severity, but UP-A was associated with a significantly greater reduction in BPD severity compared to MBT-A across the entire period.—Remission rates at 36 months were 14% in UP-A, and 7% in MBT-A.—All outcomes were reduced to a significantly greater extent in UP-A compared to MBT-A.
MBT for families									
Bateman and Fonagy 2019	UK	Outpatient	56 (29 in immediate intervention)	21 (immediate), 23 (delayed)	58% female (immediate), 62% female (delayed)	MBT-FACTS: Five evening sessions – 1st session: 1 h; remaining sessions: 1.5 h – Topics: &cir Introduction to BPD psychoeducation &cir Mindfulness and mentalizing techniques &cir Mentalizing through role plays &cir Mentalizing and empathic validation – Problem solving and review (Bateman and Fonagy 2019)	Delayed intervention	5 weeks	– Parents in immediate treatment had better outcomes than those in delayed treatment – Parents still felt significantly burdened after EOT

TABLE 2 | Overview of risk of bias in mentalization-based therapy trials.

Trial	Low risk	Unclear risk	High risk
Bateman and Fonagy 1999	✓ Selection bias (allocation concealment) ✓ Detection bias ✓ Adherence bias	△ Selection bias (random sequence generation)	✗ Incomplete outcome data ✗ Attention bias ✗ Allegiance bias
Bateman and Fonagy 2009	✓ Selection bias ✓ Detection bias ✓ Attrition bias ✓ Reporting bias ✓ Attention bias ✓ Adherence bias	—	✗ Allegiance bias
Jørgensen et al. 2013	✓ Allegiance bias	△ Selection bias △ Reporting bias	✗ Detection bias ✗ Attrition bias ✗ Adherence bias ✗ Attention bias
Laurensen et al. 2018	✓ Selection bias ✓ Detection bias	—	✗ Attrition bias ✗ Reporting bias ✗ Adherence bias ✗ Attention bias ✗ Allegiance bias
Smits et al. 2020	✓ Attrition bias	—	✗ Selection bias (skewed randomization) ✗ Detection bias (self-reported outcomes only) ✗ Adherence bias (only 42% of sessions above adequate MBT) ✗ Allegiance bias (last author involved in MBT training)
Carlyle et al. 2020	✓ Selection bias ✓ Detection bias ✓ Attrition bias ✓ Allegiance bias	△ Adherence bias (adherence ensured through supervision but no results) △ Attention bias (sessions in both arms not reported)	✗ Reporting bias (trial was registered, but in the registration 2 primary and 8 secondary outcomes are listed. Only the primary outcomes are reported in the publication)
Juul et al. 2023	✓ Selection bias ✓ Detection bias ✓ Attrition bias ✓ Reporting bias ✓ Allegiance bias	△ Adherence bias (adherence to MBT was monitored, but attendance data was missing for 21 participants, and treatment in MBT-short was not mapped in the interim period from end of treatment at five months to measurements after end of treatment in long-term MBT. Accordingly, it is unclear whether MBT/treatment in fact was short-term in the lower intensity arm)	NB: Attention bias not applicable due to trial design
Robinson et al. 2016	✓ Detection bias ✓ Reporting bias ✓ Adherence bias ✓ Allegiance bias	△ Selection bias (allocation concealment) △ Attrition bias	✗ Attention bias

(Continues)

TABLE 2 | (Continued)

Trial	Low risk	Unclear risk	High risk
Philips et al. 2018	✓ Selection bias ✓ Allegiance bias	△ Detection bias	✗ Attrition bias ✗ Reporting bias ✗ Adherence bias ✗ Attention bias
Mohajerin et al. 2024	✓ Detection bias ✓ Attrition bias ✓ Adherence bias ✓ Allegiance bias	△ Selection bias (random allocation stated, but unclear details) △ Reporting bias (no study protocol available)	✗ Attention bias (MBT group had significantly more sessions than UP group)
Rossouw and Fonagy 2012	✓ Selection bias ✓ Detection bias ✓ Attention bias	△ Adherence bias	✗ Attrition bias ✗ Reporting bias ✗ Allegiance bias
Beck et al. 2020	✓ Selection bias ✓ Detection bias ✓ Adherence bias ✓ Allegiance bias	△ Attrition bias	✗ Reporting bias ✗ Attention bias
Mohajerin et al. 2025	✓ Attrition bias ✓ Adherence bias ✓ Allegiance bias	△ Selection bias (random allocation stated, but unclear details) △ Reporting bias (protocol was registered retrospectively and is not available)	✗ Detection bias (self-reported outcomes only) ✗ Attention bias (MBT group had significantly more sessions than UP group)
Bateman and Fonagy 2019	✓ Selection bias ✓ Adherence bias ✓ Attrition bias ✓ Reporting bias	—	✗ Detection bias (self-reported outcomes only) ✗ Attention bias (compared to waitlist) ✗ Allegiance bias (authors of MBT conducted the trial)

robust and generalizable evidence regarding the effectiveness of MBT for BPD.

3.2 | Overview of Outcomes

The outcomes (both primary and secondary) used to assess the effectiveness of MBT for BPD are summarized in Table 3. A total of 46 different measures were employed across trials, with 28 instruments used only once. Most of these measures focused on symptoms related to BPD, anxiety, depression, other psychopathological symptoms, as well as general and interpersonal functioning.

4 | Discussion

This review included 14 RCTs of MBT for BPD and represents, to the authors' knowledge, the most extensive review of MBT to date. However, the findings on MBT's effectiveness remain inconclusive, with substantial heterogeneity across studies and significant limitations due to high risk of bias. Consequently, there are critical areas requiring further investigation to optimize MBT's clinical application and broaden its reach. Below, we will outline key concerns and highlight future directions.

One striking finding of this review is the predominant focus on symptom reduction as the primary outcome. While this approach is common in psychotherapy research in general (Cuijpers 2019), it becomes problematic given the limited validity and reliability of the diagnostic categories used to define BPD symptoms. The overlap between personality disorders (PDs) and common mental disorders (Krueger et al. 2014) further underscores the challenges in defining clear treatment targets and evaluating meaningful outcomes. As Cuijpers (2019, 276) aptly states, "So, if we do not yet really know what these disorders are and how they should be defined, what should be the targets of treatments and how can we measure their outcomes?"

Over the last 10 years, the PD field has tried to move toward a more precise characterization of personality pathology as reflected in the DSM-5 Section III Alternative Model for Personality Disorders and the ICD-11 framework for diagnosing PDs. Given the focus in both systems on maladaptive self and interpersonal functioning as the core distinguishing feature of PD, future research in MBT should pivot away from individual, categorically defined PDs to assess outcomes related to the core of personality dysfunction, including outcomes of personality and interpersonal functioning, such as the Level of Personality Functioning and Severity Indices of Personality Problems

TABLE 3 | Overview of the measures used in the trials to assess primary and secondary outcomes.

Measure	Number of trials	Trials
Personality disorder		
Zanarini Rating Scale for Borderline Personality Disorder (ZAN-BPD)	3	(Beck et al. 2020; Juul et al. 2023; Robinson et al. 2016)
Borderline Personality Disorder Severity Index (BPDSI)	2	(Laurensen et al. 2018; Philips et al. 2018)
Borderline Personality Features Scale for Children (BPFS-C)	2	(Beck et al. 2020; Rossouw and Fonagy 2012)
Personality Assessment Inventory-Borderline (PAI-BOR)	2	(Laurensen et al. 2018; Smits et al. 2020)
Borderline Symptom List-23 (BSL-23)	2	(Mohajerin et al. 2024, 2025)
Childhood Interview for DSM-IV Borderline Personality Disorder (CI-BPD)	1	(Rossouw and Fonagy 2012)
Structured Clinical Interview for DSM-5 Personality Disorders (SCID-5-PD)	1	(Mohajerin et al. 2024)
Personality and interpersonal functioning		
Inventory of Interpersonal Problems (IIP)	6	(Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013; Laurensen et al. 2018; Philips et al. 2018; Smits et al. 2020)
Experience of Close Relationships Inventory (ECR)	1	(Rossouw and Fonagy 2012)
Severity Indices of Personality Problems (SIPP)	1	(Smits et al. 2020)
Depression and anxiety		
Beck Depression Inventory	5	(Bateman and Fonagy 1999, 2009, 2019; Jørgensen et al. 2013; Laurensen et al. 2018)
Spielberger State-Trait Anxiety Inventory	3	(Bateman & Fonagy, 1999, 2019; Jørgensen et al. 2013)
Beck Anxiety Inventory (BAI)	1	(Jørgensen et al. 2013)
Beck's Depression Inventory for Youth (BDI-Y)	1	(Beck et al. 2020)
Depression, Anxiety, Stress Scale-21 (DASS.21)	1	(Robinson et al. 2016)
Mood and Feelings Questionnaire (MFQ)	1	(Rossouw and Fonagy 2012)
General and interpersonal functioning, attachment and quality of life		
Global Assessment of Functioning (GAF)	4	(Bateman and Fonagy 2009; Jørgensen et al. 2013; Juul et al. 2023; Robinson et al. 2016)
Social Adjustment Scale—Self-report (SAS-SR)	3	(Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013)
EuroQol (EQ-5D)	3	(Laurensen et al. 2018; Smits et al. 2020; Robinson et al. 2016)
Children's Global Assessment Scale (CGAS)	1	(Beck et al. 2020)
Short-Form Health Survey (SF-36)	1	(Juul et al. 2023)
Work and Social Adjustment Scale (WSAS)	1	(Juul et al. 2023)
Caregivers/relatives		
Borderline Personality Features Scale-Parent (BPFS-P)	1	(Beck et al. 2020)
Burden Assessment Scale (BAS)	1	(Bateman and Fonagy 2019)
Child Behaviour Checklist (CBCL)	1	(Beck et al. 2020)

(Continues)

TABLE 3 | (Continued)

Measure	Number of trials	Trials
Family Empowerment Scale (FES)	1	Bateman and Fonagy 2019)
Systemic Clinical Outcome and Routine Evaluation-15 (SCORE-15)	1	(Bateman and Fonagy 2019)
Suicidality and self-harm		
Suicide and Self-Harm Inventory (SSHI)	3	(Bateman and Fonagy 1999; Laurensen et al. 2018; Smits et al. 2020)
Risk-Taking and Self-Harm Inventory for Adolescents (RTSHIA)	2	(Beck et al. 2020; Rossouw and Fonagy 2012)
Deliberate Self-Harm Inventory (DSHI)	2	(Mohajerin et al. 2024, 2025)
Deliberate Self-Harm Inventory-9 items (DSHI-9)	1	(Philips et al. 2018)
Other psychopathology		
Symptom Checklist 90-Revised (SCL-90-R)	4	(Bateman and Fonagy 1999, 2009; Jørgensen et al. 2013; Philips et al. 2018)
Brief Symptom Inventory (BSI)	2	(Laurensen et al. 2018; Smits et al. 2020)
State-Trait Anger Expression Inventory-2 (STAXI-2)	2	(Mohajerin et al. 2024, 2025)
Adult Service Use Schedule (AD-SUS)	1	(Robinson et al. 2016)
Alcohol Use Disorders Identification Test (AUDIT)	1	(Mohajerin et al. 2024)
Eating Disorder Examination (EDE)	1	(Robinson et al. 2016)
Structured Clinical Interview for Diagnostic and Statistical Manual of DSM-5 Disorders (SCID-5)	1	(Mohajerin et al. 2024)
Timeline Follow Back (TLFB)	1	(Philips et al. 2018)
Triarchic Psychopathy Measure (TriPM)	1	(Mohajerin et al. 2024)
Warwick-Edinburgh Mental Well-Being Scale	1	(Bateman and Fonagy 2019)
Youth Self-Report (YSR)	1	(Beck et al. 2020)
Mentalization		
Mentalization: How I Feel (HIF)	1	(Rossouw and Fonagy 2012)
Reflective Functioning Scale	1	(Philips et al. 2018)
Others		
UPPS-P Impulsive Behaviour Scale (UPPS-P)	2	(Mohajerin et al. 2024, 2025)
Difficulties with Emotion Regulation (DERS)	1	(Mohajerin et al. 2025)

(SIPP). These outcomes represent the intrapsychic change associated with the psychodynamic roots of MBT, which conceptualize symptoms as manifestations of deeper personality dysfunction (Cuijpers 2019). Given that MBT aims to enhance mentalizing capacity as the driving mechanism for improved self and interpersonal functioning, these outcomes may better capture the core of what MBTs seek to change than symptom levels at a phenotypic level. Encouragingly, upcoming trials of MBT are beginning to incorporate dimensional measures such as the LPF (e.g., Hauschild et al. 2025).

Similarly, an important gap in the current MBT literature is the relatively sparse assessment of mentalizing. While MBT is explicitly designed to improve mentalizing capacity and is proposed as

the core mechanism of change, few trials have directly measured this. Addressing this gap by incorporating validated assessments of mentalizing represents a crucial direction for future research.

Another key issue is the limited generalizability of MBT findings. Most RCTs on MBT have been conducted on female-majority samples. This gender homogeneity observed in the included studies likely reflects current clinical realities where BPD is more often diagnosed in women (Leichsenring et al. 2024), and women are likewise more likely to seek and access mental health care compared to men and people of other gender identities (Seelman et al. 2017; Wendt and Shafer 2016). While inclusion and exclusion criteria in these trials do not appear excessively restrictive compared to the psychotherapy field in general, it is

TABLE 4 | Recommendations for future research and clinical practice.

Domain	Research recommendations	Clinical implications
Trial quality	Conduct high-quality RCTs with low risk of bias.	Be aware of the limitations of existing evidence when implementing MBT, and stay updated as new, higher-quality data emerges.
Outcomes	Shift primary outcomes from symptom reduction and categorical PD diagnoses to dimensional measures of personality functioning. Develop core outcome sets for personality disorder research	Routinely monitor self- and interpersonal functioning using validated measures in addition to symptom measures to assess meaningful therapeutic change.
Mechanisms of change	Conduct studies aimed at identifying active ingredients in MBT, particularly by systematically measuring changes in mentalizing and reflective functioning over the course of treatment.	Incorporate routine assessment of mentalizing or reflective functioning to track therapeutic processes and tailor interventions.
Diverse populations and settings	Expand research beyond WEIRD contexts by including trials in non-European countries. Include more gender-diverse samples, adolescents and older adults to assess age- and culture-specific adaptations.	Tailor MBT delivery to diverse populations by adapting materials and formats for different ages, genders and cultural backgrounds. Consider family involvement especially in youth populations.
Comorbidity and integrated approaches	Investigate the effectiveness of MBT in individuals with common comorbidities. Test integrated or modular MBT approaches adapted for comorbid presentations and/or conduct trials focused on dimensionally defined psychopathology to address issues of comorbidity.	Systematically screen for dimensionally defined psychopathology and/or comorbidities during assessment. Consider adapting treatment plans to include sequential or combined interventions, e.g., trauma-focused work prior to or alongside MBT.
Monitoring adverse outcomes	Increase focus on negative outcomes (e.g., dropout, non-response, symptom deterioration) and systematically report these adverse events in trials.	Monitor for adverse effects during treatment and ensure regular supervision and risk management protocols are in place.
Positioning MBT in the broader BPD treatment landscape	Compare MBT to other BPD-specific treatments in head-to-head trials, individual participant data meta-analyses, or network meta-analyses.	When selecting treatment approaches, consider patient characteristics, local resources and evidence from comparative studies. Consider being transparent with patients about the limits of current evidence.

important to consider whether participant recruitment practices or broader systemic factors may inadvertently exclude other populations. Future studies should aim to actively consider how to foster more inclusive sampling and ensure that findings are applicable to the broader population affected by BPD.

Furthermore, most RCTs on MBT have been conducted in European countries. Although two trials conducted in Iran were included, these trials represent the only non-European contributions and were both conducted by the same research group. In both trials, MBT was not found to be more effective than UP, with UP appearing more beneficial on certain outcomes. However, due to methodological concerns, including

critical details that were omitted in terms of quality of MBT training and adherence, and the limited number of studies outside Europe, no firm conclusions can be drawn about the effectiveness of MBT in non-Western contexts. These findings do, however, underscore the importance of expanding MBT research into more culturally and geographically diverse settings. Psychological treatments are often developed and tested within WEIRD (Western, Educated, Industrialized, Rich, Democratic) populations, highlighting the need to examine MBT's effectiveness beyond these contexts (Fonagy and Luyten 2021).

Furthermore, current studies predominantly focus on adults, with limited research on adolescents and older adults. BPD is

increasingly recognized in younger populations, yet we lack comprehensive evidence on the effectiveness and tailoring of MBT across the lifespan (Jørgensen, Storebø, Stoffers-Winterling, et al. 2021). Future research should include studies specifically targeting these age groups to determine age-specific needs and treatment outcomes. Another underexplored area is the role of family and caregiver involvement in MBT. Given the interpersonal nature of BPD, incorporating family-focused components may enhance treatment outcomes, yet research in this area remains scarce (Guillén et al. 2021). Similarly, BPD frequently co-occurs with other psychiatric disorders, including post-traumatic stress disorder (PTSD), substance use disorders, and eating disorders. However, the impact of these comorbidities on the effectiveness of MBT remains poorly understood. Our review identified only three RCTs that examined MBT in the context of BPD with comorbid conditions (Mohajerin et al. 2024; Philips et al. 2018; Robinson et al. 2016). In addition, secondary analyses from earlier trials suggest positive outcomes for individuals with higher clinical severity, operationalized through comorbidity (Bateman and Fonagy 2013), as well as for those with BPD co-occurring with antisocial personality disorder (Bateman et al. 2016) and trauma-related symptoms (Smits, Luyten, et al. 2022). Future research should explore whether MBT requires adaptation for common comorbid presentations and consider the development of integrated treatment approaches.

Finally, negative outcomes of MBT, including potential adverse effects, deterioration, treatment dropout and non-response, remain understudied. Psychotherapy research rarely investigates unintended consequences, yet these are critical for evaluating treatment safety and effectiveness (Cuijpers 2019; Linden and Schermuly-Haupt 2014). Standardized reporting of negative outcomes in MBT trials would improve transparency and provide a more balanced assessment of its risks and benefits. This concern is amplified by the fact that several trials included in this review reported no significant differences between MBT and comparator treatments on primary or secondary outcomes. While participants in both treatment arms frequently demonstrated meaningful clinical improvements, such as reductions in self-harm, general psychopathology and interpersonal difficulties, this raises important questions about the specific mechanisms of change in MBT. In particular, it underscores the challenge of disentangling MBT-specific effects from common therapeutic factors. Moreover, the field continues to grapple with inconsistencies in outcome selection and reporting. The wide variety of outcome measures used across trials limits comparability and may obscure patterns of differential effectiveness. This heterogeneity not only hampers the synthesis of findings across studies but also risks overestimating the robustness of MBT's effects. Greater attention to how patients actually improve—beyond statistical significance and treatment comparisons—is essential. Emerging initiatives to define a standard set of patient-reported outcomes for personality disorder research (Prevolnik Rupel et al. 2021) represent a promising development toward methodological coherence. Incorporating such standardized measures, alongside transparent reporting of both positive and negative outcomes, will be crucial in refining our understanding of MBT's true impact.

In summary, while MBT has shown promise as a treatment for BPD, research to date is limited by a high risk of bias, a narrow

focus on symptom reduction, potentially overlooking other important outcomes and limited generalizability. It is important to recognize that many of the methodological limitations identified in MBT research are not unique to MBT but reflect broader challenges across BPD treatment trials. In fact, the Cochrane review (Storebø et al. 2020) included 75 RCTs of psychological interventions for BPD, and found that, although BPD-specific treatments tended to outperform non-specific comparators, every trial was rated as having a high risk of bias due to problems in one or more domains. This underscores that the entire evidence base for BPD psychotherapies is methodologically underdeveloped. Consequently, calls for more rigorous research should extend beyond MBT to encompass the full spectrum of BPD-focused treatments, prioritizing larger samples, blinded assessments, standardized core outcomes, transparent reporting and careful attention to treatment fidelity and allegiance. By addressing some of the limitations regarding MBT trials to date, we can move toward a more comprehensive and clinically useful understanding of its impact for people with BPD. Building on these observations, we conclude by outlining several key directions to guide future research and clinical implementation of MBT in Table 4.

5 | Limitations

This review focused exclusively on RCTs of MBT to provide a more rigorous, structured and critical synthesis of the current evidence. While this approach supports clearer conclusions regarding treatment outcomes under controlled conditions, it does not capture valuable insights from non-randomized, naturalistic or qualitative studies that can enrich understanding of real-world implementation, therapeutic processes and patient experiences. Readers interested in a broader overview of the MBT literature, including such studies, are referred to Wolfs (2024).

Conflicts of Interest

MSJ, CS, and SB were involved in one of the RCTs that is included in this review (Beck et al. 2020). The quality assessment of this trial was done by independent raters. MSJ is not trained in MBT, does not practice it clinically and has no financial interests related to the method. SSH is a trained and certified MBT therapist who practices MBT therapy, supervision, training and research, but has no financial interest related to the method. CS is a trained and certified MBT therapist and supervisor and conducts training, supervision and research in MBT. SSH is a trained and certified MBT therapist and collaborates with the originators of MBT, but has no financial interest related to the method.

Data Availability Statement

Data sharing is not applicable to this article as no new data were created or analyzed in this study.

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